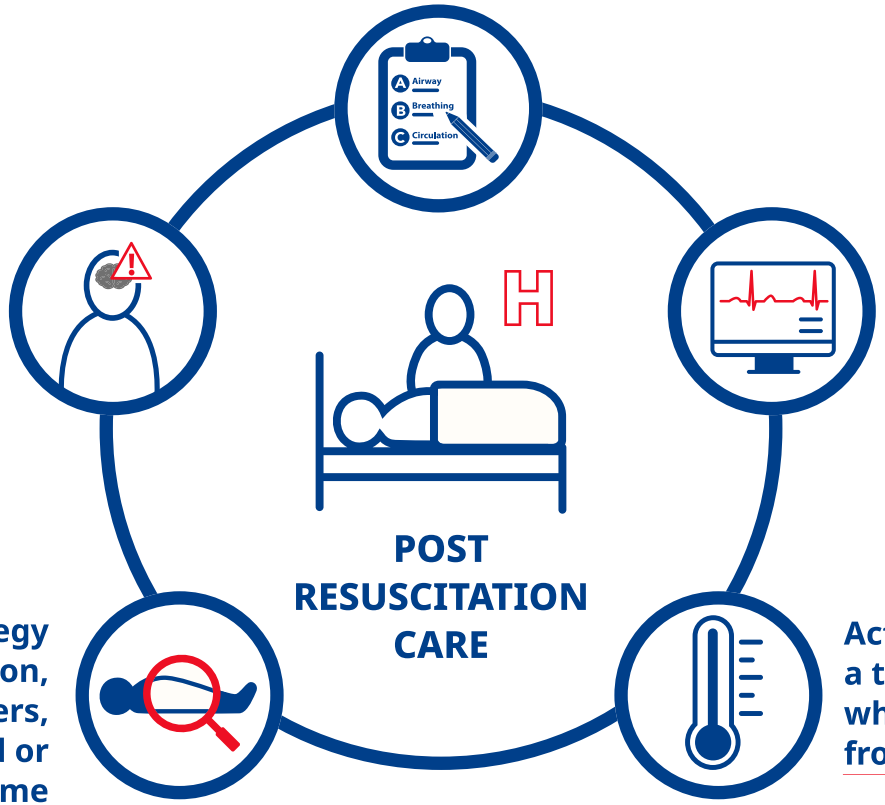
POST RESUSCITATION CARE
KEY MESSAGES

GUIDELINES 2025
EUROPEAN RESUSCITATION COUNCIL©

After ROSC use ABCDE approach

- Insert an advanced airway (tracheal intubation when skills available)
- As soon as SpO_2 can be measured reliably or arterial blood gas values are obtained, titrate the inspired oxygen to achieve an arterial oxygen saturation of 94-98%, and ventilate lungs to achieve normocapnia
- Aim for a systolic blood pressure > 100 mmHg or a mean arterial pressure > 60–65 mmHg

Perform functional assessments of physical and non-physical impairments before discharge to identify rehabilitation needs and refer to early rehabilitation if indicated



Prioritise immediate coronary angiography for patients with clear ST-elevation on the ECG or other high suspicion of coronary occlusion (e.g. haemodynamic and/or electrical instability)

Use a multimodal strategy including clinical examination, electrophysiology, biomarkers, and imaging to predict good or poor neurological outcome

Actively prevent fever by targeting a temperature ≤ 37.5 °C for patients who remain comatose after ROSC from cardiac arrest

ABCDE – airway, breathing, circulation, disability, exposure; ECG – electrocardiogram; ROSC - return of spontaneous circulation; SpO_2 - oxygen saturation